

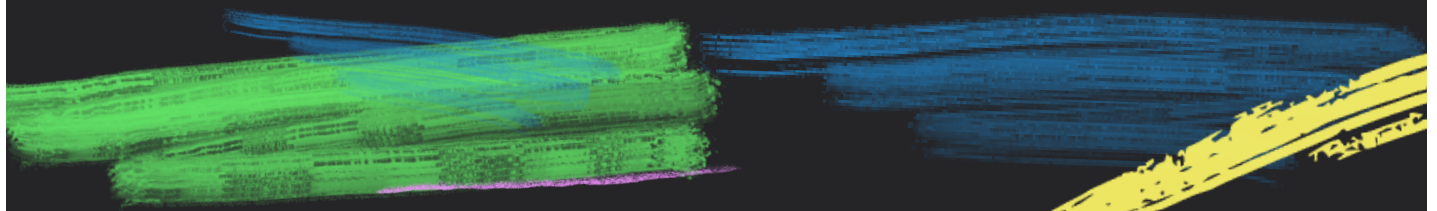


REFRESH · PUBLIC-SERVING DIVISION · REACH

Decentralized Mission Services

A companion study. How a mission that works in one market expands and extends its reach across the whole REACH network, what that kind of business is called, and how to launch it without building a directory nobody uses.

Companion study · about a 10-minute read · reads on its own, but assumes the main business case



How to use this. This companion picks up one idea from Section 7 of the main study and takes it all the way down. Read it when you want the model, the naming decision, and the launch path for letting a mission reach beyond the market it was built in. There is also a [PDF of this study](#).

CONTENTS

- | | |
|---|--------------------------------------|
| 1 What we are building | 6 What it does for the agency |
| 2 Why it could not happen before | 7 What could go wrong |
| 3 How it works, end to end | 8 The launch path |
| 4 What to call it | 9 Decisions this needs |
| 5 Who pays whom | |

SECTION 1

What we are building

Most nonprofits can only help the people who can physically reach them. An agency in Marion, Ohio can be the best in the country at moving someone from a resume to a job offer, and that excellence stops at the county line. Not because the need stops there. Because there has never been a way to deliver it anywhere else.

We are building the way. A person anywhere in the REACH network asks to be connected with a case manager on a specific topic, books a time in the app, and meets that person by video. The agency on the other end is the one that is genuinely excellent at that topic, wherever it happens to sit. It brings the expertise. Refresh brings the toolkit, the curated resources, and the person who raised their hand.

One sentence. A mission that works in one market can expand and extend its reach into every other market on the network, and Refresh runs the market that makes the match, supplies the tools, and carries the demand.

This is the same shape as telehealth, and the comparison is worth holding on to. Teladoc did not make doctors better. It made the good ones reachable, pooled demand that was too thin in any one town, and gave the encounter a place to happen. The social sector has the same thin-demand problem and none of the infrastructure.

Where this sits. This is a service line on top of the REACH network described in the main study. It only works because that network exists first: the shared commons of curated resources, the portable person account, and enough people in one place to make a specialty viable. Read **the business case and strategic launch path** for that foundation.

SECTION 2

Why it could not happen before



The limit was never talent or willingness. It was that resources reached an agency through relationships, one introduction at a time. A case manager could only refer to what they personally knew about. That worked, slowly, inside a market someone had spent years building. It could not cross into a market where nobody had made those introductions yet.

That was the governor on the whole sector. Every agency was capped at the reach of its own rolodex, so expertise stayed stranded exactly where it was built.

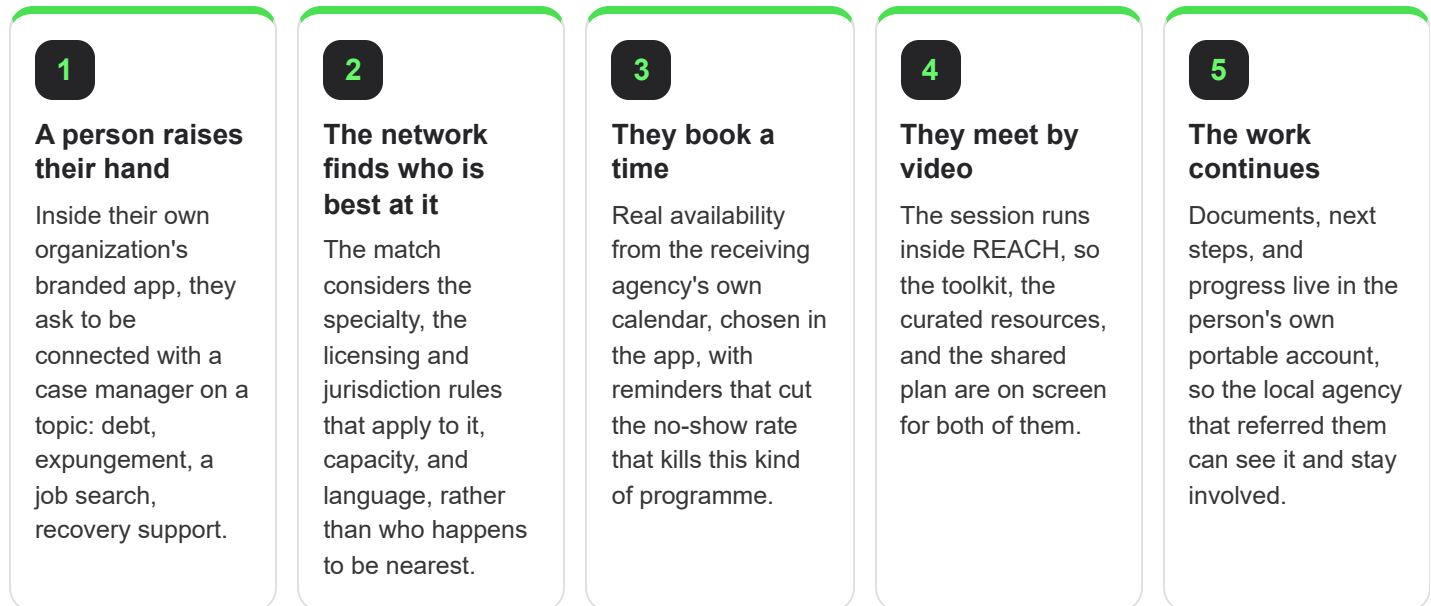
What changes. Once the commons is large, current, and searchable by proximity and by where someone is in their path, an agency no longer needs to have made the introduction. It needs only to be findable and good. The relationship stops being the distribution channel, and the network becomes the distribution channel.

Two other things had to be true, and now are. The person needs one account that travels with them rather than a file that belongs to whoever intook them, or a cross-market handoff is a dead end. And the receiving agency needs a working toolkit for the specialty, not just goodwill, or the encounter is improvised and the outcome is not defensible.

SECTION 3

How it works, end to end

Nothing here asks a person to understand the machinery. From where they sit, they clicked a button and met someone who could help.



The receiving agency is not taking the person away from whoever they came in through. The referring organization keeps the relationship and can see the work. That matters, because an agency that fears losing its people to the network will not refer into it, and the whole thing depends on referrals flowing freely.

SECTION 4

What to call it

Two different things need names here, and conflating them causes trouble later. One is the kind of business Refresh is in. The other is the kind of service the nonprofit is now selling.

What Refresh is doing

Refresh earns from running the market that makes the reach possible, and the model has several available shapes rather than one settled answer. A fee per referral the network matches. A charge for access to the tool a specialty runs on. Or pricing on the specific features a decentralized service needs, such as scheduling, video, the shared plan, or the resource toolkit. Which of these leads and which come bundled is an open decision, and the label we choose should not quietly decide it.

First choice, not decided	Alternative	Sector-native
Managed services marketplace Refresh runs the market, sets the standard, handles the match and the money, and does not employ the providers. The nearest commercial analogues are the managed marketplaces in care and legal services. It names the facilitation fee and the demand engine together.	Service enablement platform Puts the emphasis on the toolkit, which is what a receiving agency actually feels. This is how virtual-care enablement companies describe themselves. Weaker at naming where the demand comes from.	Capacity exchange Agencies contribute capacity in what they are good at and draw capacity in what they are not. Reads well to funders and boards. Reads as jargon to commercial investors.

The working choice is to describe Refresh as operating **a decentralized managed services marketplace**, and to use "enablement" as the word for the toolkit half when the audience is an agency rather than an investor. This is a first choice rather than a decision. The alternatives above stay on the page deliberately, because the people who will settle this have not weighed in yet.

What the nonprofit is selling

The Teladoc parallel is the fastest way to make this land, so the name should lean into it rather than away from it.

First choice, not decided

Telecase

Telecase management. One word, immediately legible to anyone who has used telehealth, and specific to the thing being delivered, which is case management rather than advice. Survives translation to a board, a funder, and a participant.

Alternative

Telemission

Broader, and it covers coaching, classes, and navigation as well as casework. Warmer to the sector, vaguer to everyone else, and it invites the question of whose mission.

Plain

Virtual case management

No coinage to defend and no explaining to do in a grant application. Gives up the memorability that makes the Teladoc comparison work in a first meeting.

And what the agency becomes. Whatever the service is called, the agency's role in the network has a name that already means something to the sector: a **Center of Excellence**. Marion is a Center of Excellence for Employment Development. West Virginia is one for debt management. That phrase carries the standard, the accountability, and the pride, and it survives whatever the product ends up being called.

SECTION 5

Who pays whom

The money has to make sense for three parties at once, or the market does not clear.

THE RECEIVING AGENCY EARNS

Reach it could not fund before

- Extends the mission into markets it will never physically serve
- Pooled demand makes a specialist hire viable for the first time
- Turns a cost centre into earned revenue that is not grant-restricted

THE REFERRING ORGANIZATION GETS

Coverage it cannot staff

- Access to specialties it could never justify hiring for
- Keeps the relationship and sees the work in the person's plan
- Better outcomes on programmes it is already accountable for

REFRESH EARNS

Several available shapes

- A fee per referral the network matches
- Access to the tool the specialty runs on
- Pricing on the features a decentralized service needs

Three ways the money moves

The marketplace is not one transaction repeated. It runs three at once, and they can all be live in the same specialty at the same time. Each one brings a different payer to the same encounter, which is what keeps the model from depending on any single source of money.

Business to business

One agency pays another

A Goodwill in one market pays a Goodwill in another to deliver a specialty to its participant. The agency delivering the service pays Refresh for the technology it runs on, which sits in that agency's own business plan as a cost of the service line it is now selling.

Funder-sponsored

Philanthropy buys capacity, Refresh administers it

A foundation or a fund pays Refresh to facilitate, and holds a retainer that pre-purchases slots with the providers. Refresh administers that pool, pays the providers against it, and charges a third-party administrator fee. The person is served at no cost to them.

Direct to the person

Someone applies for themselves

A person finds the service in the app and applies directly, with no agency in between. When they enroll, Refresh earns a marketing fee for having delivered the acquisition. This is the channel that scales without a partner agreement in front of it.

The three reinforce each other rather than compete. Business-to-business proves the specialty is worth paying for. Funder-sponsored capacity fills the gap for people whose agency cannot pay and who cannot pay themselves. Direct enrollment turns the network's own reach into demand that arrives without being sold. A specialty that only ever runs on one of the three is fragile; one running on all three is not.

A note on the middle one. Holding a funder's retainer and paying providers out of it makes Refresh a third-party administrator for that pool, with the obligations that role carries. It is a real business line and it is also the one with the most compliance weight, so it should be designed deliberately rather than arrived at.

Beyond those three, who funds the encounter also depends on the specialty. Some are billable against existing restricted funding. Some are an allowable programme cost for the referring organization. Some are paid by an employer or a health plan that wants its people served. And some are sponsored, where a funder buys a block of capacity in a specialty it cares about and the network distributes it where the need is.

The boundary that keeps this clean. Refresh facilitates and equips. It does not employ the case managers, does not set eligibility, and does not take a cut that varies with whether someone enrolls in a public benefit. The agency delivering the service is the accountable party for the service, exactly as it is today.

SECTION 6

What it does for the agency

The staffing problem is the one worth stating plainly, because every executive director recognises it. You cannot justify a full-time specialist for a caseload of four. You also cannot tell those four people that you do not do that here. So the work gets done badly by someone whose job is something else, or it does not get done.

Pooling demand across the network fixes both halves at once. The agency that is excellent at the specialty gets enough volume to staff it properly. Everyone else gets access to it without hiring anyone.

Before

- Expertise capped at the local market
- Specialist roles that never reach viable volume
- Referrals limited to who someone personally knows
- Revenue almost entirely restricted and granted

After

- Expertise sold into markets it will never physically serve
- One specialty staffed properly and run at scale
- Referrals arrive from the network, matched and booked
- An earned-revenue line the board actually controls

That last line is the strategic one. An agency with earned revenue in a specialty it owns is a social enterprise, not only a grantee. It has something to sell, a reason to invest in being the best at it, and a source of money that does not disappear when a funding cycle turns.

SECTION 7

What could go wrong



Four risks are worth designing against now rather than discovering later.

Quality

A bad encounter in a distant market damages the network, not just the agency. Centers of Excellence need entry standards, participant feedback that is visible, and the ability to lose the designation.

Jurisdiction

Some specialties are licensed by state, and expungement in particular is bounded by law and by unauthorised-practice rules. The match has to respect that boundary rather than route around it.

Referral fear

If an agency believes it loses people by referring into the network, it will not refer. The referring organization must keep the relationship visibly, and the data has to show it.

Thin start

A marketplace with three specialties and no volume helps nobody. The launch has to concentrate demand rather than spread it, which is the whole point of the phasing below.

SECTION 8

The launch path

The failure mode for a marketplace is launching wide and thin. Every phase here concentrates rather than spreads.

1

One specialty, two markets

Pick the specialty with the clearest expertise and the least regulatory drag, most likely employment development or debt management. One Center of Excellence, one receiving market, real people, real money.

2

Prove the encounter

Measure what a funder will ask about later: show rate, completion, outcome at ninety days, and what the receiving agency thought. Fix the toolkit against what actually broke.

3

Add markets before adding specialties

Depth first. The same specialty into four more markets builds the volume that makes the specialist hire durable. A second specialty too early splits attention and demand.

4

Open the designation

Publish the Center of Excellence standard and let agencies apply. At that point the supply side starts recruiting itself, which is the tipping point this model is built to reach.

The one number that matters early. Not agencies signed and not specialties listed. It is completed encounters per specialty per month, because that is the only figure that tells you whether the market is clearing or whether we have built a directory.

SECTION 9

Decisions this needs

Four things need an answer before the first phase can start, and three of them are naming or policy rather than engineering.

THE NAME

What the service is called to participants and to boards. The working choice is **Telecase**, with **Center of Excellence** for the agency's role regardless of what the service is named. Neither is settled, and Section 4 keeps the alternatives on the record.

THE FIRST SPECIALTY

Employment development and debt management are the two strongest candidates. Expungement is the most compelling story and the hardest first move, because of licensing.

THE MODEL

Whether Refresh charges per referral, for access to the tool, for the features a decentralized service needs, or some combination. More than one is right eventually. One is easier to sell first.

THE FIRST CHANNEL

Which of the three payment routes in Section 5 leads. Business to business is the fastest to prove. Funder-sponsored reaches the people with the least ability to pay. Direct enrollment scales furthest but needs the network to already be large.

THE STANDARD

What it takes to be designated a Center of Excellence, who decides, and what causes the designation to be lost. This is the quality guarantee the whole network rests on.